

Integrated Care for Older People – iCOPE (Plymouth & West Locality) Service specification

Specification details	
Service specification number	
Service	Integrated Care for Older People – iCOPE (Plymouth & West Locality)
Commissioner Lead	Plymouth Locality of NHS Devon
Provider Lead	Primary Care Networks in Plymouth and West
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Author(s)		Approved by	
Name and title:	Previous version: Jane Bullard – Senior Commissioning Manager Plymouth LCP Dr David Attwood - Associate Medical Director LSW	Name and title:	Edits for 26/27 Nicola Jones
Email:		Date: 16/03/2026	

1. Population needs

Current health systems are often better at responding to episodic health needs than to the more complex and chronic health needs that tend to arise with increasing age and increasing frailty. Frailty has significant impacts upon health care systems:

- Increasing numbers of people are at risk of developing frailty. A person living with mild frailty has twice the mortality risk of a fit older person.
- More people living with mild, moderate or severe frailty are attending Emergency Departments.
- Older people living with mild, moderate or severe frailty are more likely to have delayed transfers of care.
- People living with mild, moderate or severe frailty could often have their needs met best in settings outside of acute hospital care.
- Severe frailty care can cost four times the amount of caring for a person without frailty.

Older people living with frailty are at increased risk of adverse outcomes including disability, hospitalisation, nursing home admission and mortality. However, there is evidence that frailty may be a modifiable condition and appropriate intervention can influence frailty trajectories. The UK and international consensus guidance recommends identification of frailty as part of routine clinical encounters, or wider population case finding, to facilitate the planning and delivery of services for older people.

Consequently, there is a need to develop and implement comprehensive and coordinated primary health care approaches to prevent, slow down or reverse the effects of frailty in older people, and where these declines are unavoidable, help older people to compensate in ways which maximise their functional ability. Integrated-care approaches such as ICOPE are community-based, designed around the needs of the older person rather than the provider, and are coordinated effectively with long-term care systems.

The iCOPE Model assists health care professionals to detect declines in intrinsic capacity and functional ability, and to deliver effective interventions to prevent and delay the progression of frailty.

Using Population Health Management, the model segments the population on the basis of frailty severity, which dictates the management of the person and residential status, which dictates the form and function of the service.

We estimate that 22% of the PCN population aged 65 years and over have indications of frailty. Prevalence of Frailty in the pilot population aged over 65 years, by severity is:

- Mild Frailty 9.6%
- Moderate Frailty 8.3%

- Severe Frailty 4.4%

Applying the figures to the Plymouth & West Devon Locality PCN population (2026) aged over 65 years provides the following indicative frailty figures:

PCN Name	List size Jan 2026	Population 65 or over (2026)	Mild Frailty (9.6%)	Moderate Frailty (8.2%)	Severe Frailty (4.4%)	Total Mild, Moderate & Severe Frailty
Beacon Medical Group	43,435	10,170	976	834	447	2,257
Drake Medical Alliance Limited	61,154	11,437	1098	938	503	2,539
Mayflower PCN	32,701	6,790	652	557	299	1,508
Mewstone PCN	32,045	9,565	918	784	421	2,123
Pathfields Medical Group	29,659	4,970	477	408	219	1,104
Sound PCN	43,199	7,937	762	651	349	1,762
Waterside Health Network	70,283	10,766	1034	883	474	2,391
West Devon PCN	36,468	11,551	1109	947	508	2,564
Totals	348,944	73,186	7,026	6,002	3,220	16,248

2. Outcomes

2.1 NHS Outcomes Framework Domains & Indicators

Domain 1	<u>Preventing people from dying prematurely</u> Reducing deaths in babies and young children	X
Domain 2	<u>Enhancing quality of life for people with long-term conditions</u> <u>Health related quality of life for people with long term conditions</u> <u>Ensuring people feel supported to manage their condition</u> <u>Improving functional ability in people with long term conditions</u>	X X X X
Domain 3	<u>Helping people to recover from episodes of ill-health or following injury</u> Emergency admissions for acute conditions that should not usually require hospital admission	X
Domain 4	<u>Ensuring people have a positive experience of care</u> Patient experience of primary Care Patient experience of hospital care	X

	Friends and family Test <u>Improving children and young people's experience of healthcare</u> <u>Improving peoples experience of integrated care</u>	X
Domain 5	<u>Treating and caring for people in safe environment and protecting them from avoidable harm</u> Patient safety Incidents reported Reducing the incidence of avoidable harm	X

3. Scope

The aim of iCOPE is to identify people at risk of, or with frailty; keep these people as well as possible; reduce the risk of admission to hospitals or care homes for preventable causes and maximise their quality of life.

iCOPE will achieve this by:

- Delaying the onset of frailty and improve the experience of aging for older people
- Achieving the best possible outcomes for older people
- Making the most efficient use of resources
- Improving the experience of caring for older people for professional and formal and informal care providers

3.1 The iCOPE Model is intended to be both flexible and scalable and can be implemented in each PCN in a way that complements and supports existing services. **Service description/care pathway**

iCOPE includes both Frailty Identification and Frailty Interventions. In Primary Care, all older people aged 65 and over, will be assessed for frailty every 1 –2 years. If frailty is diagnosed, older people will be stratified into frailty tiers as follows:

- Fit and Well,
- Mild Frailty
- Moderate Frailty
- Severe Frailty

Interventions will be determined according to the tier of frailty, with a key focus on proactive “Ageing Well” care.

Frailty Tier - Fit and Well:

Each ‘Fit and Well’ person aged over 65 will receive Health and Wellbeing information in the form of an Annual Communication. The communication will include information to support leading a healthy, happy, older life and provide contact details of local organisations able to support healthy lifestyles.

Frailty Tier – Mild Frailty:

Each person with Mild Frailty aged over 65 will receive an Annual Communication from their GP practice / PCN, which includes information to reverse frailty or prevent / delay the progression of frailty.

- Referral to the PCNs Social Prescriber who will offer goal-based interventions promoting wellbeing and social connectedness, and health / lifestyle advice
- Referral to Falls Classes
- Osteoporosis Risk Assessment

The Annual Health Care Communication has three themes:

1. Staying Connected, including digitally connected
2. Staying Active
3. Staying Healthy

The suggested Annual Communication details will be provided to PCNs and will have a strong emphasis on behavioural change and social prescribing. The evidence for this comes from a systematic review of Behavioural Change Techniques (BCT) in older people in the BJGP¹. This review found that significant improvements in the quality of life of older people were seen when Social Prescribers discussed and undertook the following Behavioural Change Techniques with older people:

- Goal setting
- Action planning
- Problem solving
- Social support
- Instructions on how to perform a behaviour
- Health consequences

Frailty Tier – Moderate or Severe Frailty:

People with moderate or severe frailty will receive / have access to:

- A full Comprehensive Geriatric Assessment (CGA) provided by PCN Medical Professionals and Livewell Southwest Ageing Well MDT Team. The CGA is a holistic assessment including both medical and functional elements. The output of the CGA process is a Care Plan for each person, which will be shared with healthcare providers across the Integrated Care System footprint (including OOGH Providers).

People with moderate or severe frailty will be offered a full Comprehensive Geriatric Assessment (CGA) provided by PCN GPs or other suitably qualified professionals with similar expertise and the Livewell Southwest Ageing Well MDT Team. If clinical input is not available through the PCN, discussions should take place with the Community Provider who may be able to provide additional support.

The CGA is a holistic assessment including both medical and functional elements. The output of the CGA process is a Care and Support Plan (akin to the 'This is me' document) for each person, which will be shared with healthcare providers across the Integrated Care System footprint (including OOH Providers). The CGA involves assessing and optimising the following to support patients to achieve their goals:

1. Assessment of Patient Goals
2. Long term conditions review focussing on optimising symptoms
3. Medications review with a focus on deprescribing unnecessary medications
4. Assessment of mood and memory

¹ Ahmed, S. et al Behaviour change techniques in personalised care planning for older people: a systematic review British Journal of General Practice 2021;71(703): e121-e127. DOI: <https://doi.org/10.3399/bjgp20X714017>

5. Advance care planning
6. Assessment of activities of daily living (social circumstances, activities of daily living, continence, nutrition, skin, nutrition, mobility and falls, hearing and vision)

CGA software has been developed by Pathfields Medical Group on SystmOne and will be available to all PCNs in the Plymouth & West locality free of charge and without any future licence fee. Alternatively, the CGA software that each Practice is currently using as part of the EHCH National Service Specification can be used.

It is expected that, on average, there will be a **minimum** of 2 CGAs completed per week per 30,000 population. This figure will be averaged per quarter and the expected figure per PCN is shown below:

PCN Name	Total minimum number of new CGAs 26-27	Quarterly target 26-27
Beacon Medical Group	151	38
Drake Medical Alliance Limited	212	53
Mayflower PCN	113	28
Mewstone PCN	111	28
Pathfields Medical Group	103	26
Sound PCN	150	37
Waterside Health Network	244	61
West Devon PCN	126	32
Totals	1210	302

If the minimum number is not achieved in each quarter there will be an opportunity to catch up in next quarter. If after one year the minimum number is not achieved or if significantly fallen behind ICB may recover funding. The LMC will be engaged in liaison with PCN if this situation occurs.

An Urgent Visiting Service operating at PCN level and closely aligned to the Community Crisis Response Team (CCRT). Each PCN will have an ARP / Early Responder who will conduct early morning urgent home visits. Visits will be undertaken for patients with moderate or severe frailty and the following conditions:

- Abdominal pain
- Breathlessness
- Falls
- Acute confusion
- Acute immobility or functional decline eg new incontinence or neglect
- Clinician judgement indicates that a patient requires an urgent visit

Timely transportation to hospital should, following assessment by a visiting service, secondary care testing be required.

3.2 Identification of Frailty

Practices have access to validated frailty identification tools such as the Electronic Frailty Index (eFI) and will be using these tools as part of the 2017-2019 Frailty GMS contract and to identify patients who would benefit from the Structured Medication Review as per the updated GP contract.

However, eFI does have limitations and may produce a high false negative and false positive diagnostic rate for frailty. As an alternative to the eFI, SystmOne practices can use the Pathfields Tool. The Pathfields Tool cannot currently be used by EMIS Practices. Work is underway to develop a method of streaming the Pathfields Tool to all EMIS Practices. Further information / details on Frailty Identification and Coding is shown at Appendix 1.

3.3 Clinical sessions to support iCOPE

iCOPE Clinical Session funding is available on the basis of 3 clinical sessions per week for a PCN with a population of 30,000 This should encompass:

- 1 MDT session led by a GP or clinician with appropriate expertise in older people
- 1 clinical session led by a GP or clinician with appropriate expertise in older people
- To action the outputs of the MDT (such as long-term conditions review, medications deprescribing, advance care planning)
- Support to MDT from duty team if MDT colleagues undertaking visits
- Osteoporosis risk assessment, investigation, and management in patients with mild Frailty

3.4 Wellbeing resources

Resources will be put in place to ensure that all areas of Plymouth and West Devon have access to Wellbeing services. Additional Wellbeing Services to support iCOPE are shown at Appendix 2.

3.5 Population covered

iCOPE will be provided by PCNs to the registered GP provider Network Population or GP provider population aged 65 years and over².

A small number of people who are younger than 65 may well be considered suitable for the iCOPE approach and it is up to PCNs/Practices to decide whether a younger patient would benefit from a Comprehensive Geriatric Assessment (CGA).

If PCNs consider that patients outside the 65 years and over age group would benefit from the iCOPE approach, discussions should be held with the ICB in the first instance.

3.6 Any acceptance and exclusion criteria thresholds

Acceptance

- Patients aged 65 and over

3.7 Interdependence with other services/providers

The provider will work in partnership with Livewell Southwest MDT Ageing Well Teams, Integrated Care Partnership providers, and Local Authorities (Plymouth City Council and

² Gale C., et al Prevalence of frailty and disability: findings from the English Longitudinal Study of Ageing Age and Ageing, Volume 44, Issue 1, January 2015, Pages 162–165, <https://doi.org/10.1093/ageing/afu148>

Devon County Council) with regard to Wellbeing and Support Services provided by the VCSE.

4. Applicable Service Standards

- 4.1 Proactive Care – Providing Care and Support for People Living at Home with Moderate or Severe Frailty
<https://www.england.nhs.uk/long-read/proactive-care-providing-care-and-support-for-people-living-at-home-with-moderate-or-severe-frailty/>

5. Applicable quality requirements and CQUIN goals

Applicable Quality Requirements (See Schedule 4 Parts [A-D])

Each PCN / Practice must ensure that any personnel involved in providing any aspect of care under ICOPE has the necessary training, skill and competency to do so. Audit and Incident requirements are described below:

Formal incidents should continue to be reported to NHS England using the SEA form:-

- <https://www.england.nhs.uk/south/wp-content/uploads/sites/6/2021/08/2021-22-sea-incident-report-form.docx>

Patient safety events can be recorded by staff members from your own and other organisations or members of the public on to Learning from Patient Safety Events LfPSE about your or other organisations' practice.

To see and respond to these incidents you will need to be registered with LfPSE.

LfPSE allows users to access data that has been submitted to better understand their local recording practices and culture, and to support local safety improvement work. Registering and reporting onto LfPSE and learning from incidents demonstrates to Care Quality Commissioner (CQC) that your organisation has a positive safety culture.

GP practices should have received a letter from NHS Devon Integrated Care Board providing information on LfPSE. If you would like any face-to-face support with registering, please contact the PSIRF inbox – d-icb.PSIRF@nhs.net

6. Key Service Outcomes

Outcomes and KPIs will be monitored quarterly. These Outcome Measures should be recorded using the Read Codes shown at Appendix 1.

Service Outcomes	Outcome Measure
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Frailty Identification / Diagnosis	• Number of Patients identified as Fit and Well (i.e. not Frail) • Number of Patients identified with Mild Frailty • Number of Patients identified with Moderate Frailty • Number of Patients identified with Severe Frailty • Number of Patients (with frailty) who are Housebound • Number of Patients (with frailty) resident in Care Homes
Frailty Register (subdivided by residential status)	
Contact with Patients who are Fit and Well (ie not Frail)	• Number of Patients that are sent an Annual Communication
Management of Patients with Moderate or Severe Frailty	• Number of Patients who had a CGA (minimum of 2 per week per 30,000 population)

7. Financial requirements

iCOPE is a Local Enhanced Scheme. Funding for this LES will be in respect of:

- Identification and Recording of Patients with / at risk of developing Frailty
- Annual Communication with 65+ 'Fit and Well and Mild Frailty' Population
- Additional information provided to individuals with mild frailty Eg Links to Social Prescribing / Falls classes
- Clinical Sessions and representation at practice MDT

PCNs will be paid on a monthly basis and is calculated based on the below total funding split into 12 equal payments over the year (applying a push payments model). Payments are subject to PCNs submitting quarterly reports, as detailed in Section 6. Key Service Outcomes.

Total funding available for each PCN is shown below:

PCN	25/26 (the payments for 26/27 will be calculated according to any agreed inflation and relevant list size)	Monthly allocation 25/26 (26/27 to be confirmed per note re. annual payments)
Beacon Medical Group	£97,524.24	£ 8,127.02
Drake Medical Alliance	£123,547.56	£ 10,295.63
Mayflower Medical group	£87,381.60	£ 7,281.80
Mewstone	£75,490.92	£ 6,290.91
Pathfields	£65,001.48	£ 5,416.79
Sound	£92,991.48	£ 7,749.29
Waterside	£136,824.72	£ 11,402.06
West Devon	£83,975.64	£ 6,997.97

Totals	£762,737.64	£ 63,561.47
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DDRB will be used as the start point for annual 'inflationary' review, with any deviation from DDRB uplift requiring clear rationale.

8. Location of provider premises

The Provider's (GP) Premises.

9. Evaluation

Service review will happen annually in October with a comprehensive review during 26/27. All PCNs that are signed up to iCOPE will be expected to engage with and support the review process.

References

- 1 Ahmed, S. et al Behaviour change techniques in personalised care planning for older people: a systematic review British Journal of General Practice 2021;71(703): e121-e127. DOI: <https://doi.org/10.3399/bjgp20X714017>
- 2 Gale C., et al Prevalence of frailty and disability: findings from the English Longitudinal Study of Ageing Age and Ageing, Volume 44, Issue 1, January 2015, Pages 162–165, <https://doi.org/10.1093/ageing/afu148>

Appendix 1 – Identification of Frailty and Coding

1. PCNs/Practices that have batch added the eFI frailty codes to frailty diagnosis read codes must batch delete and start again.

To clarify there are two codes:

- a) eFI read codes: “Electronic Frailty Index score” (SNOMED code: 1084961000000105)) and “FI- Frailty Index” (SNOMED code: 713636003)
- b) Frailty read codes: “severe frailty” (SNOMED code: 925861000000102) and “moderate frailty” (SNOMED Code: 925831000000107)

Practices that have batch added “eFI severe frailty” → diagnosis severe frailty must delete these. Same is true for moderate and mild frailty if this has taken place. This will result in patients being incorrectly diagnosed as frailty (less than half of these patients have frailty), **which threatens patient safety.**

2. PCNs/practices are to use a method of their choice to identify patients with frailty and sub-categorise by residential status using the read codes below (this will help with payment, the EHCH spec, and potentially the anticipatory care spec).
3. Information governance has been signed off by the Data Protection Officer (DPO) for primary care and DPO for the ICB
4. PCNs/practices to communicate with all patients with mild frailty a about the following:
 - a) Offering a referral to social prescriber
 - b) Offering a referral to falls class
 - c) Information on healthy living, remaining active and independent and links to organisations that can support this in their locality.
 - d) Advise on the importance of digital connectivity. In the COVID pandemic having older people connected to internet and smart phones has been invaluable in terms of providing healthcare information and better care. Suggest to all older people that they ask their families, social prescribers, or local libraries to support with this and provide the practice with updated emails and phone numbers

Predictive modelling indicates that a network of 30,000 patients will have 450-500 patients living with mild frailty or moderate frailty and not housebound, who would benefit from this communication.

Service outcomes:

iCOPE 01 Frailty identification	
Rationale for inclusion	Maintaining a dynamic frailty register is excellent practice and separates Western locality from the rest of the nation. It allows the locality to be much more responsive to the needs of the patients as and when they slip into crisis
Numerator	Number of Patients identified as Fit and Well (ie not Frail): • SystemOne: Read code "Frailty assessment" (1086611000000106) AND "Unsuitable for frailty assessment" (1086631000000103) • EMIS: Read code "[RFC] well person" (Concept ID 90761000006102;Description ID 90761000006118) Number of Patients identified with Mild Frailty: • Read code "Frailty assessment" (1086611000000106) AND "Mild frailty" (925791000000100) Number of Patients identified with Moderate Frailty: • Read code "Frailty assessment" (1086611000000106) AND "Moderate frailty" (925831000000107) Number of Patients identified with Severe Frailty: • Read code "Frailty assessment" (1086611000000106) AND "Severe frailty" (925861000000102) Number of Patients (with frailty) who are Housebound: • Read code "housebound" SNOMED 160689007 Number of Patients (with frailty) who live in a care home: • Read code "lives in care home" SNOMED 248171000000108
Denominator	• PCN list size • PCN list size (age 65+)

iCOPE 02: Annual communication with patients who are fit and well	
Rationale for inclusion	As a locality we want to promote healthy ageing and prevent or delay the onset of frailty.
Numerator	Number of patients who are fit and well that are provided with an annual communication that contains lifestyle advice, information on social prescribers, and physical activity classes • Number of Patients offered a referral for to a Social Prescriber: Read code “social prescribing offered” SNOMED 871691000000100 • Number of Patients offered a referral to a Falls Class: Read code “referral for physical activity service offered” SNOMED 767621000000102 • Number of Patients given Lifestyle advice: Read code “lifestyle education SNOMED 313204009
Denominator	Patients 65+ who are fit and well: • Read code “Unsuitable for frailty assessment” (1086631000000103) in SystmOne • Read code “[RFC] well person” (Concept ID 90761000006102;Description ID 90761000006118) in EMIS

iCOPE 03A: Patients with mild frailty and lifestyle interventions	
Rationale for inclusion	Many patients with mild frailty are at the early stages of the condition and more focussed support could reverse frailty or delay progression. They need more targeted support.
Numerator	Number of patients with mild frailty that are provided with an annual communication that contains lifestyle advice, information on social prescribers, and physical activity classes Number of Patients referred to social prescribing service Read code: SNOMED 871731000000106

	Social prescribing declined Read code: SNOMED 871711000000103 Number of Patients referred to a Falls Class: - Read code referral for physical activity service offered" SNOMED 767621000000102 Number of Patients given Lifestyle advice: - Read code "lifestyle education" SNOMED 313204009
Denominator	Patients with mild frailty: Read code: "Mild frailty" SNOMED 925791000000100

iCOPE 03B: Patients with mild frailty – assessing for osteoporosis

Rationale for inclusion	Many patients with mild frailty are at the early stages of the condition and more focussed support could reverse frailty or delay progression. Anecdotally the "herald event" for progression is an injurious fall and a fracture, starting the cycle of loss of confidence, fear of falling, and reduced mobility. A risk assessment for osteoporosis and starting bone sparing drugs when indicated should reduce fracture risk with time (NNT ~14-20 to prevent L spine fracture and 91 to prevent a hip fracture over three years)
Numerator	Number of patients with a FRAX score or if not clinically appropriate (e.g. 90+ years old) a bone health assessment: - Read code "Bone health assessment" SNOMED 965891000000102 - Read code "WHO FRAX 10 year osteoporotic fracture probability score" SNOMED 736956002
Denominator	Patients with mild frailty: Read code "Mild frailty" SNOMED 925791000000100

iCOPE 04: Patients with moderate-severe frailty - CGA	
Rationale for inclusion	Patients with moderate to severe frailty benefit from proactive CGA with emerging data showing evidence of improved independence, survival, and a reduction in healthcare utilisation in Western locality. However, to deliver this benefit CGAs must conform to the minimum quality standards as outlined in the Healthy Ageing Commissioning Handbook endorsed by Devon ICS. Firstly, it needs to be conducted by a multidisciplinary team (MDT). Secondly the CGA process should follow three “I” stages: • Information gathering stage • Intervention stage • Information sharing. Care and support plans should be offered to the patient, they should contain the information outlined in the previous section (as appropriate), and (with consent) shared with healthcare providers in the locality If either these three steps are not taking place, <u>or</u> if an MDT review is not undertaken, then a CGA cycle is not being delivered.
Numerator for CGA	Number of Patients who had a CGA: • Read code “subject of comprehensive geriatric assessment plan SNOMED 836131000000104
Numerators for sharing the care and support plan	Number CGAs shared with patient OOH GP and Paramedics: • Read code: “sharing advance care planning decisions with out of hours service” SNOMED 922301000000104
Numerators for advance care planning	Number of patients with an advance care plan: • Read code “treatment and escalation plan” SNOMED 735324008. NB. Ensure that this is shared on Summary Care Record so paramedics and UHP can see
Numerators for TEP forms and	Number of patients with a Treatment Escalation Plan on DCCR read codes are:

resuscitation status	• Treatment Escalation Plan SNOMED CT 735324008 and add 'free text' associated with the code 'TEP in Devon and Cornwall Shared Care Record'. If you wish to add resuscitation decision please use these read codes: a. Not for attempted CPR (cardiopulmonary resuscitation) (SNOMED 450476008) b. For attempted cardiopulmonary resuscitation (SNOMED 450475007) •
Numerators for hospitalisation status	Number of Patients with a hospital admission decision documented: • For future hospital admissions on TEP: read code "listed for admission to hospital" (SNOMED 183767005) OR • Not for future hospital admissions on TEP: read code "patient refuses hospital admission" or "hospital admission declined" SNOMED 183960004
Numerators for medication reviews	Number of patients with a structured medication review: • Read code: "structured medication review" SNOMED 1239511000000100
Denominator	Patients with moderate or severe frailty read code and who DO NOT live in a care home: • Read code "Moderate frailty" (925831000000107) • Read code "Severe frailty" (925861000000102) AND NOT • Read code "lives in care home" SNOMED 248171000000108

Appendix 2 – iCOPE Wellbeing Resources



Age Positive
Overview Flyer (5).pdf

Downloadable flyer for GP – hard copies available

(i) An older people Social Prescribing post

A specialist post, working within the network of social prescribers across PCNs, focussing on older people, to gather information on what is available for older people and support the PCN social prescribers to link people into these services. This Social Prescriber will map activity, ensure accessibility and help to create further activities.

(ii) Additional time banking capacity focussing on older people

Through iCOPE, Time Banking capacity will be doubled enabling a recruitment drive to increase membership from the over 65s and linking older people into activities at the Wellbeing Hubs or through other community venues.

(iii) Peer support co-ordination to develop peer support offer across the Wellbeing hubs focussed on LTCs and carers

A Peer Support Coordinator will be employed to set up peer support across the Wellbeing Hubs and other community venues. The aim is to link people into groups that take place in their community, creating connections between people, building confidence and self-esteem so that people are able to maintain independence for longer. Each peer support group will agree a programme of social, educational or other activities which it will aim to take forward with support from the Wellbeing Hub.

The Peer Support Coordinator will work with existing providers such as the Caring for Carers service, Long-term Conditions self-management programmes and Health Coaches/Social prescribers in primary care, to identify older people who would benefit from ongoing peer support. This would also enable upskilling of people to lead Peer Support Groups and activities, creating a sustainable way for Peer Support to thrive.



Falls Referral form -
HCP

Appendix 3 – Falls referral form

Falls Prevention Team Referral Form (For Non-Physiotherapists)	
Email your Referral to: LIVEWELL.therapyreferrals@nhs.net	
Referral form will be returned if all sections are not completed. This is NOT a referral form for the UHP Consultant Falls Clinic – refer to Devon Formulary	
Inclusion Criteria - Aged 55 years and over and a Plymouth resident - Experienced one or more falls in the last 12 months OR near miss falls OR are fearful of falling PLUS - They are interested to gain benefits from exercise (group and at home self-monitored) They are independently able to: - Walk around their own home and outdoors with or without a walking aid - Access the community without assistance from another person to walk and can access transport - Undertake activities of daily living (though may be finding them more challenging) - Follow simple instructions and monitor their own effort/challenge	☐ ☐ ☐ ☐ ☐ ☐ ☐ Exclusion Criteria - Under-55 - Requires physical assistance to access the community - Unable to participate in exercise for medical reasons/contraindications* see page 3 Ongoing medical investigations that preclude starting exercise immediately e.g., cardiac investigations - Unable to manage activities of daily living independently e.g., MDT management required - Wheelchair user Reliance on four wheeled walker or zimmer frame e.g., unable to achieve 30 seconds in unsupported stand - Vestibular diagnosis with no other multifactorial or balance problems - Residents in care homes - Neurological condition – liaise with the falls prevention team directly prior to making a referral (livewell.fallstherapyteam@nhs.net)
I confirm to my knowledge that no exclusion criteria are present ☐ *If the criteria are not met/exclusions are present please complete the appropriate referral form to the correct service.	
<u>Personal Information</u>	
Person's name: NHS no: Date of birth:	
Address (include permanent address if temporary):	
Tel:	
Permission to contact NOK: Yes ☐ No ☐	
NOK name and contact details:	
GP name and surgery:	

Referrers Details: *(by completing this section you are confirming the person has consented to the referral)*

Date of referral:

Referred by (name, position, location):

Tel. no:

Email:

Reason/s for Referral: *(What does the individual want to improve/get back to?)*

Triage Questions *(the individual will be assessed by the most appropriate professional within the team e.g., physiotherapist or exercise specialist)*

Is the individual experiencing any of the following?

- | | | | |
|--|--------------------------|------------------------------|--------------------------|
| Marked asymmetry | ☐ | *Unexplained falls | ☐ |
| Complex balance disorder, including unmanaged vestibular disturbance markedly impacting on everyday function | ☐ | New pain | ☐ |
| A new diagnosis, e.g., diabetes, angina that requires 1:1 monitoring of exercise | ☐ | Unmanaged visual disturbance | ☐ |
| A sudden deterioration of physical function | ☐ | Breathlessness ++ | ☐ |
| None of these | ☐ | | |

***If the person has experienced recurrent falls, unexplained falls, a long lie, or loss of consciousness (or suspected) please confirm that any possible medical causes of falls have been assessed/investigated (we do not have access to medical support within the team):** Yes ☐ No ☐

By whom: GP ☐ Geriatrician ☐ Advanced practitioner ☐ Other ☐ *Please state:*

What was the outcome of the investigations/interventions? *(e.g., lying/standing BP, cardiac assessment)*

Link to World Falls Guidelines: <https://www.bgs.org.uk/wfg>

Relevant Medical History: *For GP's please attach GP summary ☐

Medication: *For GP's please attach GP summary ☐

We aim to complete all assessments in Mount Gould Hospital Local Care Centre, or another community-based venue.

Is there a clinical need for the individual to be seen at home? Yes ☐ No ☐ (please state):

Are there any known lone worker risks to staff? Yes ☐ No ☐ (please state):

***Absolute Contraindications to Exercise (from exclusion criteria)**

- **Uncontrolled** angina
- Resting Systolic BP > 180 mmHg or resting Diastolic BP > 100 mmHg
- Tachycardia > 100 bpm
- Significant, unmanaged postural hypotension
- Acute systemic illness (e.g., acute cancer-related problems, pneumonia)
- **Unstable** or acute heart failure (including **recent** myocardial infarction or ECG changes, that have not yet been medically managed)
- Recent **injurious** fall **without** a medical examination

Appendix 4 – Patient Journey Map

Fit and Well – Patient Journey



Mr. Stanmore

Mr Stanmore is 67 and lives with his wife, 63, in Ivybridge. He manages no ongoing health conditions, however, has felt some social isolation following his retirement last year.

Mr Stanmore's is classified as "Fit & Well" with no signs of Frailty.

Just prior to Mr Stanmore's 68th Birthday, he receives a "Birthday Card" from his practice.

Mr Stanmore submits feedback to his practice about his general health and wellbeing. He also finds links to "Active Devon" and registers interest in a local walking group.

Mr Stanmore has become a member of several groups locally, including a weekly cycling group and walking football. He reports good levels of general health and wellbeing and the increase in exercise has strengthened muscles and is no longer under his local physios for Knee Problems and has avoided onward Orthopaedic referral. He is feeling less isolated and feels much more connected to his local community.

Mr Stanmore's practice, and therefore the wider system, receive data to support local interventions and healthcare decision making.

Need to access other primary or secondary care services is reduced; e.g., Mental Health services and Primary Care Orthopaedics. Mr Stanmore reduces risk of moving up on the Frailty Index.

Mild Frailty – Patient Journey



Mrs. Weston

Mrs Weston is 73 and lives alone in Leigham in Plymouth. She lives with Type 1 diabetes which she manages well.

Mrs Weston attended for routine bloods at her practice. The HCA noted she has some difficulty mobilising from her seat. The nurse asked Mrs Weston more about her mobility and how she has been feeling. Mrs Weston admitted she has been struggling more than usual lately, both physically and mentally. The HCA discussed her options and Mrs Weston agreed to have future conversations in order to provide further support, the HCA noting observations on the system.

Mrs Weston's is identified by her practice as having Mild Frailty, using the Pathfields Tool. An appointment is arranged for Mrs Weston to see the practice's social prescriber, and they decide together that joining a local crafting group may help Mrs Weston feel less isolated and get her out and about in the community. She also reported suffering with loneliness following losing her husband last year and has since become a frequent attendee at practice, presenting with low mood and depression several times in the last few months. Together they decided a referral to the Bereavement Counselling service may be helpful.

In the weeks following her appointment with the social prescriber, Mrs Weston gained confidence and felt happier in herself. She was enjoying becoming more social and decided to additionally join a local Tai Chi class.

She has not attended practice for low mood since referral to the Bereavement Counselling Service. Mrs Weston reported feeling much better mentally and stronger physically.

Just prior to Mrs Weston's 73rd Birthday, she receives a "Birthday Card" from her practice. She returns her questionnaire to practice and reads about a local charity Timebank; she is introduced to Mary, 77 who also lives in Leigham. Mrs Weston donates time to teach Mary to cross-stitch, and "banks" time with Mary to help her improve her crochet skills.

Mrs Weston's physical and mental wellbeing has significantly improved following VCSE support and guidance. She has grown her support network, meeting Mary through Timebank has gained her a trusted friend in the community and she reports feeling much less alone these days. Tai Chi has helped her strength and she now takes walks every day.

She has been making less frequent appointments with her GP and is feeling hopeful for the future.

Mrs Weston's practice receives a wide range of feedback and data which can then be used to help the evaluation of the iCOPE project and understand areas for improvement and celebrate successes.

Moderate Frailty – Patient Journey



Mrs. Lawrence

Mrs Lawrence is 75 and lives with her eldest daughter in the Devonport area of Plymouth. She is seeing District Nurses weekly for dressing changes.

She has COPD and Stage 2 Parkinson's.

She has recently been diagnosed with Moderate frailty but is at high risk of this becoming severe.

The Frailty Nurse practitioner conducts an in-depth assessment (CGA) with Mrs Lawrence. She reports a recent and rapid decline in her mobility and now relies on her Daughter for all personal care. On good days, she can walk as far as the kitchen from the lounge, and some days she feels unable to mobilise at all. Mrs Lawrence is feeling frustrated and feels she is losing her independence, and guilty for relying on her daughter, which is raising tensions in the home.

Her daughter is unable to push her in a wheelchair over the door threshold, so Mrs Lawrence's only leaves the house when her Grandson can come over to help. Mrs Lawrence reports to often having a low mood and feelings of isolation.

She reports she has taken the same medication for years (6 different pills per day) however both her and her daughter are unsure what they are for.

Mrs Lawrence reports feeling significant pain daily.

Mrs Lawrence's Frailty Nurse Practitioner (FNP) suggests a range of interventions for Mrs Lawrence, such as Pain Management referral, support from local charities, a review with a pharmacist for medication advice and an OT referral to help with adaptations to the home. She also advised she may be eligible for building works to help in aiding her pass the threshold.

The FNP provides Mrs Lawrence's daughter a leaflet for "Caring for Carers" with a view of helping her feel supported in her role as carer.

Mrs Lawrence reports feeling much happier, even just to have had someone to talk to about her problems.

Together, they set realistic goals centered around what means most to Mrs Lawrence.

She hopes to manage to get out of the house once a week and perhaps make friends in the community. She would like to be able to mobilise more often, and perhaps undertake some personal care without the help of her daughter.

Mrs Lawrence's case is discussed at MDT.

Mrs Lawrence is referred to the practice Pharmacist and undergoes a medication review. It is identified that Mrs Lawrence is still on anti-sickness tablets following an episode of vertigo 20 months previously, which is contraindicated for those suffering with Parkinson's. An OT assessment is booked, and the GP refers on to Pain Management. Recent weight loss has been highlighted which prompted a dietitian review.

She continues to be discussed at subsequent MDTs to monitor her progress.

Outcome

Adaptations were made to Mrs. Lawrence's home so that she is now able to get out and about in her wheelchair. She reports far less pain and is much steadier on her feet. She feels she has many more good days than bad days. She has had no falls or admissions to hospital since intervention began. Her nutrition has greatly improved, and she has gained some weight.

Home tensions have improved greatly, as her daughter has support in the community and Mrs. Lawrence can now manage a level of personal care.

Mr Northcott's CGA was shared with herself and her daughter.

A combined patient record and shared IT has meant Mrs. Lawrence information is available to those who need it.

A wide range of data is also available around reduction in admissions, measurable improvement in quality of life, medication usage and VCSE service access which is shared. This is invaluable in aiding the evaluation of the iCOPE project and helps to identify areas of improvement and celebrate successes.

Signs of deterioration are looked out for at her at various points of contact with her practice. Any concerns or observations are then available to be discussed at MDT, and she remains on the agenda for the coming months.

Any new information can be gathered, new goals set and interventions put in place.